Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 07-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase 4, Single-Arm, Open-Label Study to Evaluate Safety, Tolerability, and Efficacy of Mavacamten in Adults With Symptomatic Obstructive Hypertrophic Cardiomyopathy in India **Short Title/Acronym:** ROVER Study (A Study of Mavacamten in Adults With Obstructive Hypertrophic Cardiomyopathy in India) **Posting ID:** 975158337838081384 **Protocol Number:** CV027-1146 **NCT Number:** NCT07004972 **Trial Status:** RECRUITING **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Mavacamten (BMS-986427) **Trade Name:** Camzyos **ATC Code:** C01EB24 **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/camzyos-epar-product-information_en.pdf **Phase of Development:** Phase 4 **Medical Monitor:** Bristol Myers Squibb Clinical Trials Medical Monitor

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and effectiveness of mavacamten in adults with symptomatic obstructive hypertrophic cardiomyopathy in India. **Secondary Objectives:** To evaluate the proportion of participants with an improvement of at least one New York Heart Association (NYHA) functional class at Week 30 compared to baseline, and to track cardiovascular events including major adverse cardiac events, CV hospitalizations, and heart failure events.

2.2 Background & Rationale To address the clinical need and gather local real-world safety, tolerability, and efficacy data for mavacamten, a cardiac myosin inhibitor, in the Indian population diagnosed with symptomatic obstructive hypertrophic cardiomyopathy (HCM).

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label (None) **Randomization:** N/A

3.2 Planned Enrollment & Duration Targeted Enrollment (N): 50 participants **Number of Sites:** Multiple clinical sites across India **Estimated Study Start:** 04-Sep-2025 **Estimated Primary Completion:** 13-Dec-2027

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosed with obstructive hypertrophic cardiomyopathy (HCM) consistent with current American College of Cardiology Foundation/American Heart Association and European Society of Cardiology guidelines. 2. Has unexplained LV hypertrophy with nondilated ventricular chambers in the absence of other cardiac (ie, hypertension, aortic stenosis) or systemic disease and with maximal LV wall thickness $\ge$ 15 mm (or $\ge$ 13 mm with positive family history of hypertrophic cardiomyopathy). 3. Has LVOT (Valsalva left ventricular outflow tract) peak gradient $\ge$ 50 mmHg during screening as assessed by TTE at rest, Valsalva maneuver, or post exercise LVOT peak gradient. 4. Has LVOT peak gradient with Valsalva at screening TTE of $\ge$ 30 mmHg. 5. Has adequate acoustic windows to enable accurate TTEs. 6. Has New York Heart Association (NYHA) Class II or III symptoms at screening. 7. Body weight is greater than 45 kg at screening. 8. Documentation of LVEF $\ge$ 55% at rest on screening TTE.

4.2 Exclusion Criteria 1. Known infiltrative or storage disorder causing cardiac hypertrophy that mimics obstructive hypertrophic cardiomyopathy, such as Fabry disease, amyloidosis, or Noonan syndrome with LV hypertrophy. 2. Has paroxysmal atrial fibrillation present per the investigator's evaluation of the participant's ECG at the time of screening. 3. Has persistent or permanent atrial fibrillation not on anticoagulation for at least 4 weeks prior to screening and/or not adequately rate controlled within 6 months prior to screening. (Note: Participants with persistent or permanent atrial fibrillation who are anticoagulated and adequately rate-controlled are allowed). 4. Has a history of syncope with exercise within 6 months prior to screening. 5. History of sustained ventricular tachyarrhythmia (> 30 seconds) within 6 months prior to screening. 6. Has documented obstructive coronary artery disease (> 70% stenosis in one or more epicardial coronary arteries) or history of myocardial infarction. 7. Other protocol-defined Inclusion/Exclusion criteria apply.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Mavacamten administered orally at specified doses on specified days based on patient response and protocol. **Route of Administration:** Oral **Duration of Treatment:** Up to 48 weeks

5.2 Concomitant & Prohibited Therapy Permitted: Standard-of-care baseline medications for comorbidities, including proper anticoagulation and rate-control for permitted AF. **Prohibited:** Concurrent medications strongly impacting CYP metabolism (e.g., strong CYP2C19/CYP3A4 inhibitors or inducers) or drugs contraindicated with Mavacamten.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, TTE, LVOT gradient, NYHA class assessment, LVEF measurement
Baseline	Day 0	First Dose, Baseline Safety Labs
Interim	Week 30	NYHA functional class evaluation, Safety Labs, Efficacy Assessment
End of Study	Week 48	Final Safety Evaluation, Incidence of TEAEs collection, Exit Interview

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of serious Treatment Emergent Adverse Events (TEAEs) Up to Week 48. **Measurement Method:** Standard clinical safety monitoring and reporting.

7.2 Secondary & Exploratory Endpoints - **Endpoint 1:** Proportion of participants with an improvement of at least one NYHA functional class at Week 30 compared to baseline. - **Endpoint 2:** Number of participants with major adverse cardiac events. - **Endpoint 3:** Number of participants with CV hospitalization. - **Endpoint 4:** Number of participants with heart failure (HF) events.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard ongoing safety and tolerability assessment focusing on Treatment Emergent Adverse Events (TEAEs) through Week 48. **Serious Adverse Events (SAEs):** Any severe TEAEs are documented and reported within 24 hours per regulatory guidelines. **Data Monitoring Committee (DMC):** Internal sponsor-directed monitoring to ensure continuous safety review and LVEF safety tracking.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving $\ge$ 1 dose) for TEAE evaluation; Intent-to-Treat (ITT) population for NYHA efficacy and cardiovascular event endpoint analysis. **Sample Size Justification:** $N=50$ participants to provide adequate safety and tolerability data within the specific Indian sub-population. **Handling of Missing Data:** Methods such as Last Observation Carried Forward (LOCF) for NYHA class status and missing safety data evaluations per sponsor SAP.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular clinical onsite and remote monitoring visits to ensure protocol adherence and patient safety. **Audit Trail:** Electronic Case Report Form (eCRF) tracking, data locking, and automated query resolution.

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11. Study Identifiers & Governance

Primary ID: CV027-1146 **Registry Number:** NCT07004972 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** ROVER (A Study of Mavacamten in Adults With Obstructive Hypertrophic Cardiomyopathy in India) **Official Regulatory Title:** A Phase 4, Single-Arm, Open-Label Study to Evaluate Safety, Tolerability, and Efficacy of Mavacamten in Adults With Symptomatic Obstructive Hypertrophic Cardiomyopathy in India

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12. Clinical Framework (Hypertrophic Cardiomyopathy Specific)

Disease Areas: Symptomatic Obstructive Hypertrophic Cardiomyopathy **Disease/Syndrome Name:** Hypertrophic Cardiomyopathy **Preferred UMLS Name:** Obstructive hypertrophic cardiomyopathy **Semantic Type:** Disease or Syndrome **Medical Definition:** A form of CARDIAC MUSCLE disease, characterized by left and/or right ventricular hypertrophy (HYPERTROPHY, LEFT VENTRICULAR; HYPERTROPHY, RIGHT VENTRICULAR), frequent asymmetrical involvement of the HEART SEPTUM, and normal or reduced left ventricular volume. Risk factors include HYPERTENSION; AORTIC STENOSIS; and gene MUTATION; (FAMILIAL HYPERTROPHIC CARDIOMYOPATHY). **MeSH Code:** D002312 **HPO Codes:** HP:0001639 **SNOMED ID:** 233873004 **Diagnosis Threshold:** LVOT peak gradient $\ge$ 50 mmHg; LVEF $\ge$ 55% **Medical Methodology:** Targeted cardiac myosin inhibition via Mavacamten (Camzyos) **Optimization Target:** Relief of functional obstruction and symptomatic improvement (NYHA class)

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13. Study Procedures & Methodology

Management Pathway: Clinical monitoring of heart failure symptoms via NYHA classification and echocardiography. **Education Protocol (HCP):** Investigator training on mavacamten titration, TTE evaluation, and LVEF monitoring criteria. **Education Protocol (Patient):** Informed consent, drug adherence instructions, and symptom tracking. **Quality Audit Mechanism:** Routine source data verification and clinical data management audits.

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14. Observation & Follow-Up Schedule

Total Duration: Up to 48 weeks **Onsite Visit Cadence:** Regular visits scheduled for Baseline, Week 30, and Week 48 (alongside titration safety checks) **Remote Monitoring Frequency:** Intermittent per site standard operations **Checklist Review Interval:** Routine continuous assessment for Treatment Emergent Adverse Events

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]